

SAME SIZE ARTWORK
LEAFLET SIZE: 140 mm x 88 mm

ulcers of the leg, it causes delayed wound healing and increased liability to infections. SUPRICORT should not be applied to any skin crease areas. SUPRICORT should be used with caution near the eyes and should be used for short courses only. Application to the eyes has produced corneal ulcers, raised intraocular pressure, and reduced visual function. The treatment of psoriasis with SUPRICORT may provoke the pustular form of the disease. 12. RECOMMENDED DOSAGE, DOSAGE SCHEDULE AND ROUTE OF ADMINISTRATION: Simple Topical Application: The effective action of Corticosteroids is not more than 6 to 8 hours when applied locally. There is usually a need for three applications per day in acute cases. In resolving chronic dermatoses, a single application is usually adequate. SUPRICORT Cream contains the most potent corticosteroid and the thinnest application possible should be used. Occlusive and Plastic Dressing: Wash and disinfect the skin. Apply a small quantity of Cream and massage. Apply a new layer of the medication but do not massage. Cover the medicated area with a plastic sheet and fasten to skin. For dry or scaly skin, insert a moist linen between the medication and sheet and make the so called moistened plastic dressing. It is advisable to remove the plastic sheet and take a bath at least once in 24 hours. 13. SYMPTOMS AND TREATMENT FOR OVERDOSAGE AND ANTIDOTE (S): Topically applied corticosteroids can be absorbed in sufficient amounts to produce systemic effects. 14. PACKING/PACK SIZES : Tube of 15 gm. Also available in jar of 250 gm. 15. STORAGE CONDITION Store below 30°C. Protect from light. 16. SHELF LIFE 3 years DATE OF REVISION 1 October 2018		For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory supricort™ Cream 0.025% w/v SPACE FOR PHARMACODE
Manufactured by: glenmark PHARMACEUTICALS LTD. Plot No. E-37, 39, D-Road, MDC, Satpur, Nashik - 422 007, Maharashtra State, India. TM Trade Mark	Product Registration Holder: Glenmark Pharmaceuticals (Malaysia) Sdn Bhd (660397-W), D-31-02, Menara Suezcap 1, No. 2, Jalan Kerinchi, 59200 Kuala Lumpur, Malaysia PE00000 MY	

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PHARMACOKINETICS: The extent of percutaneous absorption of topical corticosteroids is determined by many factors including the vehicle, the integrity of the epidermal barrier, and the use of occlusive dressings. Topical corticosteroids can be absorbed from normal intact skin. Inflammation and/or other disease processes in the skin increase percutaneous absorption. Occlusive dressings substantially increase the percutaneous absorption of topical corticosteroids. Thus, occlusive dressings may be a valuable therapeutic adjunct for treatment of resistant dermatoses. Once absorbed through the skin, topical corticosteroids are handled through pharmacokinetic pathways similar to systemically administered corticosteroids. Corticosteroids are bound to plasma proteins in varying degrees. Corticosteroids are metabolized primarily in the liver and are then excreted by the kidneys. Some of the topical corticosteroids and their metabolites are also excreted in the bile. 7. TOXICOLOGY: Carcinogenesis, Mutagenesis, and Impairment of Fertility Long-term animal studies have not been performed to evaluate the carcinogenic potential or the effect on fertility of topical corticosteroids. Pregnancy Category-C Corticosteroids are generally teratogenic in laboratory animals when administered systemically at relatively low dosage levels. The more potent corticosteroids have been shown to be teratogenic after dermal application laboratory animals. There are no adequate and well-controlled studies in pregnant women on teratogenic effects from topically applied corticosteroids. Therefore, topical corticosteroids should be used during pregnancy only if the potential benefit justifies the potential risk to the fetus. Drugs of this class should not be used extensively on pregnant patients, in large amounts, or for prolonged periods of time. Nursing Mothers It is not known whether topical administration of corticosteroids could result in sufficient systemic absorption to produce detectable quantities in breast milk. Systemically administered corticosteroids are secreted into breast milk in quantities not likely to have a deleterious effect on the infant. Nevertheless, caution should be exercised when topical corticosteroids are administered to a nursing woman. 8. INDICATIONS: SUPRICORT Cream is indicated in all inflammatory dermatoses. It is often effective in more difficult and resistant dermatoses where other topical corticosteroids have failed. The indications for SUPRI-CORT Cream include: - Atopic Eczema - Infantile Eczema	- Besnier's Prurigo - Discoid Eczema - Varicose Eczema - Psoriasis - Seborrheic Dermatitis - Contact Dermatitis - Neurodermatitis - Lupus Erythematosus - Erythema Multiforme - Pyodermatitis - Impetigo - Otitis Externa - Ano-genital Pruritus - Senile Pruritus 9. CONTRAINDICATIONS: Corticosteroids have been shown to be teratogenic in animals following dermal application. As agents are absorbed percutaneously teratogenicity following topical application cannot be excluded. Therefore SUPRICORT should not be used during pregnancy. SUPRICORT is contra indicated in the treatment of herpes simplex, vaccinia or varicella. Long-term use is contra indicated in patients with diabetes mellitus or tuberculosis. SUPRICORT should not be used on infants and young children. If irritation develops, topical corticosteroids should be discontinued and appropriate therapy instituted. 10. SIDE EFFECTS/ADVERSE REACTIONS: The following adverse skin reactions have been reported with the use of topical steroids: dryness, burning, itching, local irritation, striae, skin atrophy, atrophy of subcutaneous tissues, telangiectasia, hypertrichosis, change in pigmentation and secondary infection. Adrenal suppression has also been reported following topical corticosteroid therapy. Posterior subcapsular cataracts have been reported following systemic use of corticosteroids. 11. DRUG INTERACTION: None reported. 12. PRECAUTIONS/WARNINGS: Treatment should be discontinued if unfavourable reactions are seen. Regular review should be made of the necessity for continuing therapy. If a secondary microbial skin infection is present suitable concomitant antimicrobial therapy should be instituted. SUPRICORT should not be used to treat infections and
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ICONGRAPHICS CODE

PANTONE SHADE  PANTONE BLACK PROCESS C
Supersedes Artwork Code